CMS Proposed Rule: Interoperability Standards & Prior Authorization for Drugs Comments

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1. **Applicability to QHPs (Non-FFE/SHOP)**

It is unclear whether the rule applies to QHPs not offered on the Federally Facilitated Exchange or SHOP. This creates uncertainty in compliance planning and resource allocation. CMS should clarify applicability for off-exchange individual and small group plans.

2. **Interaction with State Requirements (Michigan SB 247)**

The relationship between the CMS proposal and existing Michigan electronic prior authorization requirements is unclear, including whether federal requirements expand beyond current state mandates. CMS should clarify alignment and any preemption of state requirements.

3. **Implementation Timeline**

The proposed timeline may not allow sufficient time for system development, vendor coordination, and testing. Additional time may be needed to ensure readiness and compliance.

4. **Technology & Cost Burden**

Significant system modernization is required, including API development and workflow redesign. These changes are expected to increase vendor costs and internal IT investment.

5. **Prior Authorization Workflow & Configuration Impact**

Differences in prior authorization timelines across Medicare and QHP products will require system reconfiguration. Plans may need to separate QHP and large group products within shared tools, increasing operational complexity.

6. **Fraud, Waste & Abuse Risk**

Greater reliance on automated approvals may reduce documentation rigor and increase risk of inappropriate approvals, leading to increased costs and reduced oversight.

7. **Lack of Auto-Approval Clarity**

The rule does not define criteria for automated approvals, which may lead to inconsistent implementation and operational risk.

8. **Metric Reporting Concerns**

Public reporting of authorization metrics may be misinterpreted and could create unintended incentives to alter utilization management practices.

9. **Vendor Dependency**

Pharmacy implementation depends on PBM capabilities, while medical workflows require additional EHR enhancements, limiting direct control over readiness.